National Cancer Control Programme (NCCP) 00419 · clinician draft

Carboplatin AUC 2 with radiotherapy

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Six weekly carboplatin AUC 2 visits · head and neck context

Active medicines: Carboplatin

Emetogenic risk: moderate

Medicine	Dose / route	When
Ondansetron	16 mg PO OD	Day 1 Source-linked draft; reconcile to one authorised prescription before use.
Dexamethasone	8 mg PO OD	Day 1 Source-linked draft; reconcile to one authorised prescription before use.
Dexamethasone	8 mg PO OD	Days 2-3 Source-linked draft; reconcile to one authorised prescription before use.
Metoclopramide	10 mg PO TDS	PRN Source-linked draft; reconcile to one authorised prescription before use.

Additional phase-specific support

- Carboplatin AUC 2 intravenous (IV) every 7 days for 6 cycles with radiotherapy. This is the current NCCP 00419 v6 head and neck schedule; do not substitute the AUC 1.5 seven-cycle post-induction pathway.
- Coordinate each weekly treatment with the radiotherapy and recovery plan. No automatic induction, adjuvant fluorouracil or maintenance phase is added by this regimen.
- No routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) or extra supportive-care prescription is specified by NCCP for this phase. Review marrow recovery and infection history individually.
- Arrange dental/oral assessment, mouth care, dietetic and swallowing support according to need. Review weight, oral intake, pain and hydration throughout chemotherapy and radiotherapy; no automatic feeding-tube decision is generated.
- Preserve the CUH high/moderate and docetaxel proforma adjuncts where applicable: omeprazole, local nystatin/chlorhexidine mouth care, breakthrough antiemetics and loperamide. These are historical local reference items requiring current-policy and patient-specific review, not universal prophylaxis or an additional duplicate prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00419 v6 · pNCCP 00419: treatment/supportive-care sections; NCIS v8 pp4–5 · NCCP 00419 with current NCIS and local supportive-care policy

Sources checked 2026-11-10 · SACTCheck v0.84.48 · Regimen-level drafts; prescriber and oncology-pharmacy verification required